

PATIENT: BLACKWELL, THEODORE R DOB: 09/14/1957 MRN: E4085221
DATE OF SERVICE: 05/12/2026 PROVIDER: Anita Deshpande, MD, FACC

REASON FOR CONSULT

Follow-up of atrial fibrillation and heart failure with reduced ejection fraction (HFrEF), medication titration.

HISTORY OF PRESENT ILLNESS

Mr. Blackwell is a 68-year-old man with persistent atrial fibrillation on apixaban, HFrEF (LVEF 35% on echo 05/10/2026), stage III sigmoid adenocarcinoma on FOLFOX, CKD stage 3, and type 2 diabetes. He reports mild exertional dyspnea, stable 2-pillow orthopnea, no PND. Weight up 1.5 kg over two weeks. Denies chest pain, syncope, palpitations. Chemotherapy tolerated with mild fatigue; oncology aware of cardiac status.

MEDICATIONS REVIEWED

Apixaban 5 mg BID; metoprolol succinate 100 mg daily; sacubitril-valsartan 49-51 mg BID; spironolactone 25 mg daily; empagliflozin 10 mg daily; furosemide 40 mg daily (increased today to 40 mg BID); metformin 1000 mg BID.

EXAM

BP 108/68, HR 74 irregular, O2 98% RA. JVP 9 cm. Irregularly irregular rhythm, II/VI systolic murmur at apex. Trace bilateral pitting edema. Lungs with faint bibasilar crackles.

ASSESSMENT AND PLAN

1. HFrEF, LVEF 35% - mild congestion; increase furosemide to 40 mg BID, daily weights, repeat BMP in 1 week given CKD. Continue guideline-directed medical therapy (GDMT).
2. Atrial fibrillation, rate-controlled - continue apixaban 5 mg BID and metoprolol.
3. CHA2DS2-VASc 4. No cardioversion planned while on active chemotherapy.
4. Cardio-oncology - monitor for oxaliplatin-related vasospasm; troponin and NT-proBNP q2 cycles. NT-proBNP today 1450 pg/mL (prior 1620).
4. Follow up 4 weeks or sooner if weight increases >2 kg. Patient advised to seek ED evaluation for chest pain, syncope, or worsening dyspnea.

Electronically signed: Anita Deshpande, MD, FACC – Lakeview Heart Institute